INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nivolumab, ipilimumab, pemetrexed and cisplatin

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
42 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00714
Version 4

Treatment stages

01 **Cycle 1 · chemotherapy on Days 1 and 22**
Two chemotherapy visits

02 **Cycle 2 onwards · immune continuation**
Alternating immune visits

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	6
03	Symptoms and supportive care	13
04	Sources and further information	23

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Follow your oncology team's emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

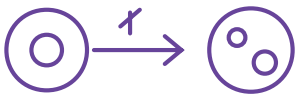
Three medicines. Different jobs.

This treatment aims to control metastatic non-small-cell lung cancer, help symptoms and extend life. It is usually given without an expectation of cure. The team checks the cancer subtype, molecular results, your health and your preferences before recommending this particular chemotherapy backbone.

IMMUNOTHERAPY

Nivolumab

Opdivo



Blocks the PD-1 immune checkpoint

Nivolumab binds PD-1 on T cells. It prevents signals that can reduce the immune attack on cancer. This can also let immune cells inflame healthy tissues. It is infused every 21 days in this plan.

T cell → PD-1 blocked → Immune response

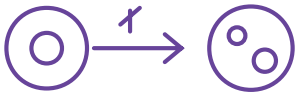
What to watch for

New breathing, bowel, skin, hormone or other symptoms need prompt advice during or after treatment. Seek urgent help for new chest pain, severe muscle weakness, drooping eyelids or difficulty swallowing or breathing.

IMMUNOTHERAPY

Ipilimumab

Yervoy



Blocks the CTLA-4 immune checkpoint

Ipilimumab blocks CTLA-4, another control on T-cell activity. It works alongside nivolumab through a different immune checkpoint. It is given every 42 days, so it is absent from each Day 22 visit. Healthy organs can also become inflamed.

T cell → CTLA-4 blocked → Immune activity

What to watch for

Report diarrhoea, tummy pain, rash, severe tiredness, headache or other new symptoms promptly. Keep your immune alert card.

CHEMOTHERAPY

Pemetrexed

Alimta



Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA)/RNA building

Pemetrexed blocks enzymes needed to make DNA and RNA building blocks. Dividing cancer cells are affected, but healthy blood-forming and mouth-lining cells can also be injured. Prescribed folic acid and B12 reduce toxicity; do not omit them because pemetrexed is an antifolate. There are two planned doses in this regimen.

DNA building blocks → Supply disrupted → Cell division slows

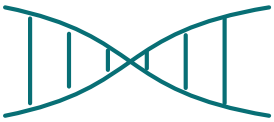
What to watch for

Low blood counts, sore mouth, rash, sickness and kidney problems can occur.

CHEMOTHERAPY

Cisplatin

Platinum chemotherapy



Damages DNA

Cisplatin forms links in DNA that hinder copying and repair. Healthy tissues can also be harmed. Kidney injury, changes in salts in the blood, hearing damage and nerve symptoms are important risks. Extra fluids and electrolyte checks are part of treatment. There are two planned doses in this regimen.

DNA → Platinum links → Copying disrupted

What to watch for

Report reduced urine, vomiting, ringing in the ears, hearing change or numbness. Hydration instructions are individual.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this combination is offered

What do the cancer terms mean?

Metastatic means cancer has spread beyond where it started. non-small-cell lung cancer (NSCLC) is a broad lung-cancer group; non-squamous describes one histology, often adenocarcinoma. First-line means the first systemic treatment for this setting. The pathology and molecular results help determine whether this exact pemetrexed combination suits your cancer.

What does the evidence mean for me?

In CheckMate 9LA, the strategy of two immune medicines with a short chemotherapy course helped people live longer than chemotherapy alone. The study included different histologies and platinum medicines. It cannot tell exactly what benefit you will have, or prove that this particular platinum is better for you.

What are the alternatives?

Ask how tumour tests, fitness and your priorities affect the options, including other drug combinations, targeted treatment when relevant, a clinical trial or symptom-focused care. Support for breathlessness, pain, appetite and emotional concerns can accompany anticancer treatment.

Why this treatment is offered

The combined strategy improved survival compared with chemotherapy in the trial population, with some durable responses. Benefits and harms vary between people.

The study tested the combined strategy across different lung-cancer histologies and chemotherapy choices. Your own expected benefit depends on your cancer and health.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Induction → review → maintenance

Treatment stages

01 **Cycle 1 · chemotherapy on Days 1 and 22**
Two chemotherapy visits

02 **Cycle 2 onwards · immune continuation**
Alternating immune visits

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TWO CHEMOTHERAPY VISITS

Cycle 1 · chemotherapy on Days 1 and 22

C1 · D1 | C1 · D22

The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and cisplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle.

ALTERNATING IMMUNE VISITS

Cycle 2 onwards · immune continuation

C2 · D1 | C2 · D22 | C3 · D1 | …

Nivolumab every 21 days; ipilimumab every 42 days. No further chemotherapy. Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Checks across both phases

Relative stages, not equally spaced dates. Ask who books and reviews each check. Do not enter dates to interpret this guide.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00714 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab, ipilimumab, pemetrexed and cisplatin: treatment and checks

First-line metastatic non-small-cell lung cancer (NSCLC) · confirm pemetrexed-compatible histology

The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and cisplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle. Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Cycle 2+ · Day 1	Cycle 2+ · Day 22	Completion / follow-up
Treatment NCCP 00714	Confirm indication, histology and plan	All four medicines	Nivolumab + pemetrexed + cisplatin; NO ipilimumab	Nivolumab + ipilimumab; NO chemotherapy	Nivolumab only; NO ipilimumab or chemotherapy	Stop at progression/toxicity or up to 24 months; agree follow-up
Blood counts, kidney and liver NCCP 00714	Baseline full blood count (FBC) and renal/liver profile	Review before treatment	Review before second/ final chemotherapy	Review before every Day 1 dose	Review before every Day 22 dose	Continue unresolved -toxicity checks per plan
Glucose, thyroid and immune symptoms NCCP 00714	Glucose, thyroid function tests (TFTs), morning cortisol; hepatitis B surface antigen (HBsAg)/ hepatitis C virus (HCV) RNA	Glucose/ thyroid function tests (TFT) before this 42-day cycle; immune symptom review	Immune symptom review; extra tests if indicated	Glucose/ TFT before every 42-day cycle; symptom review	Immune symptom review; extra tests if indicated	Symptoms can occur at least five months after last dose

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Cycle 2+ · Day 1	Cycle 2+ · Day 22	Completion / follow-up
Vitamins and steroids NCCP 00714	Folate lead-in; B12 in preceding week; reconcile steroid plan	Daily folate; rash/ antiemetic medicines as prescribed	Final planned pemetrexed: repeat prescribed rash/ antiemetic course	No further pemetrexed: complete 21-day folate tail; review other support	No automatic B12/rash steroids or induction antiemetics	Agree stop dates and ongoing medicines; no abrupt steroid taper
Platinum precautions NCCP 00714	Renal, hearing and nerve review; hydration/ electrolyte plan	Review hydration, electrolytes and renal function	Review counts, kidney, hearing and nerves again	Platinum has stopped; review persisting effects	Platinum remains stopped	Follow any residual kidney, hearing or nerve effects

What each check is for

1 Baseline and first treatment

Before Cycle 1 Day 1

Confirm suitability and preventive medicines

Ask / act: Confirm results, prescriptions, contacts and the actual chemotherapy backbone.

2 Final chemotherapy visit

Cycle 1 Day 22 (21 days after first visit)

Two chemotherapy administrations complete the planned induction

Ask / act: Check next phase and stop/review chemotherapy support; continue folate for 21 days after final pemetrexed.

3 Immune continuation

Cycle 2 onwards, Days 1 and 22

Different medicines and support at alternating visits

Ask / act: Review joint immune hold rules and duration; maintain alert-card education.

4 End of treatment

At progression, unacceptable toxicity or up to 24 months without progression

Delayed immune effects and unresolved toxicities still need care

Ask / act: Provide responsible-team, monitoring and urgent-contact arrangements.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Alternating visits and duration

STEP 2

Time in the Day Ward

Nivolumab over 30 minutes first; ipilimumab over 30 minutes on Day 1 only; pemetrexed over 10 minutes; cisplatin over 60 minutes, starting 30 minutes after pemetrexed, with pre/post hydration. Ipilimumab requires vital signs every 30 minutes during infusion and for one hour afterwards. Clinic time also includes assessment, preparation, observation and support.

STEP 3

Medicine and monitoring appointments

Folic acid starts before pemetrexed, continues through both chemotherapy visits and for 21 days after the last pemetrexed dose. B12 is given in the preceding week. The source repeats B12 every three cycles if pemetrexed continues; this planned course has only two pemetrexed administrations, so do not automatically continue B12 in immune-only cycles. Your team reconciles the steroid used to prevent pemetrexed rash with the anti-sickness steroid into one clear prescription. Do not add the two schedules together or use leftover steroids yourself.

STEP 4

Transport and access

intravenous (IV) treatment through agreed venous access; no home chemotherapy pump. Arrange transport, lab appointments and prescriptions. Allow extra time for pre/post hydration and review fluid restrictions, urine and electrolyte needs individually. Keep immune alert information available for every healthcare contact.

Before the first visit

The preventive medicines and baseline review are part of the treatment.

Describe your usual health

Tell the team about your usual breathing and bowel pattern, autoimmune illness, lung inflammation, infections, nerve or hearing symptoms, diabetes, and all prescription or non-prescription medicines. This helps identify a new problem promptly.

Prepare contact and practical support

Keep your oncology and out-of-hours instructions, a thermometer, treatment names and immune alert card available. Arrange blood tests, transport and help at home where needed. Ask what to do if a support medicine is missed; do not double a dose without advice.

Two chemotherapy visits, then continuing immunotherapy

Day 1 and Day 22 differ

Cycle 1 Day 1 includes all four medicines. Day 22 is three weeks later and includes nivolumab plus the second and final planned chemotherapy doses, but no ipilimumab. A 42-day cycle is six weeks.

The next 42-day cycles

From Cycle 2, Day 1 includes nivolumab and ipilimumab. Day 22 includes nivolumab only. No pemetrexed or platinum is planned in this continuation phase. The support medicines change too; check the updated prescription.

How long does treatment last?

Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review. The team reviews benefit, symptoms and scans. A delay can allow investigation or recovery. It does not mean you have failed treatment.

What the checks are looking for

Different checks assess safety today, possible toxicity and cancer response.

Blood count, kidney and liver

Blood counts and kidney/liver tests are checked before each Day 1 and Day 22 treatment. Blood counts show infection-fighting cells, oxygen-carrying cells and platelets. Kidney results influence chemotherapy safety. Normal results from a previous visit do not rule out a new problem.

Hormones and immune effects

Glucose and thyroid tests are checked before each 42-day cycle. The baseline assessment includes morning cortisol and hepatitis tests. New symptoms can require extra tests between planned visits; routine blood tests alone cannot exclude all immune side effects.

Scans and decisions

Your consultant sets the scan schedule. The team considers symptoms, examination, results and what matters to you. Side effects are not a reliable measure of how well treatment is controlling cancer.

Care after a drug stops

Stopping chemotherapy or immunotherapy changes the plan but does not immediately remove all risks.

After the second pemetrexed dose

Finish the prescribed 21-day folic-acid tail after the last pemetrexed. Review anti-sickness and rash-steroid stop dates. B12 is not automatically continued just because immune therapy continues.

After the last immune dose

Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose. Keep the alert card and tell other healthcare professionals about both immune medicines. Ask who will arrange follow-up and review any unresolved symptoms.

Use your team’s urgent instructions

When you call, state your treatment names, last treatment date, what changed, temperature if known, and whether you can drink and pass urine. Do not delay calling while gathering every detail.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day 1 visit	 **Day 1** Includes ipilimumab; chemotherapy in Cycle 1 only
Day 22 visit	 **Day 22** Nivolumab; chemotherapy in Cycle 1 only; no ipilimumab
Early sickness effects	 **Days 1–5** After the first chemotherapy visit; timing varies
Sickness after second chemotherapy	 **Days 22–26** Cycle 1 only; symptoms can last longer
Counts / energy / mouth	 **Days 1–42** Chemotherapy effects vary; no day guarantees recovery
Immune symptoms	 **Days 1–42** Any day, including after treatment ends

One 42-day induction cycle: The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and cisplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle.

No guaranteed symptom-free days: The symptom bands are broad examples. Counts and side effects do not recover to a fixed calendar; immune symptoms can begin after treatment ends.

Later cycles contain immunotherapy only: Day 1: nivolumab + ipilimumab. Day 22: nivolumab alone. Chemotherapy-related support is reviewed at the phase change.

Care after treatment: Folic acid starts before pemetrexed, continues through both chemotherapy visits and for 21 days after the last pemetrexed dose. B12 is given in the preceding week. The source repeats B12 every three cycles if pemetrexed continues; this planned course has only two pemetrexed administrations, so do not automatically continue B12 in immune-only cycles. Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose.

> **Why these bands stay broad:** This is a 42-day reference cycle. Only Cycle 1 has chemotherapy; Day 22 never includes ipilimumab. Symptom bands illustrate possible periods, not daily risk or a prediction of recovery. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose. Do not wait for the next appointment if new or serious symptoms develop.

Between-treatment notes

Day 1: Day 1 visit

Nivolumab and ipilimumab are due. In Cycle 1, pemetrexed and cisplatin are also due. Checks, preparation and observation add to the infusion time. Report symptoms before treatment and confirm your support prescription.

Day 2–21: Between the first and second visits

Follow the prescribed support plan after chemotherapy. Symptoms can occur at any time; do not use the date to decide they are safe. Contact your team for new diarrhoea, breathing changes, fever or sudden illness.

Day 22: Day 22 visit · no ipilimumab

In Cycle 1 this is nivolumab and the second/final chemotherapy administration. In later cycles this is nivolumab only. Blood count, kidney and liver checks remain due. Confirm the active medicines and support plan; do not repeat ipilimumab at this visit.

Day 23–42: Before the next 42-day cycle

After the final pemetrexed, folate continues for 21 days. In immune-only cycles, continue to report symptoms between visits. Confirm the next Day 1 appointment and glucose/thyroid as well as usual blood checks.

Support around the medicines

1 Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Use generic high-risk NCIS row including olanzapine. Cisplatin requires pre/post hydration and electrolyte review; follow individual/local fluid policy. Folic acid starts before pemetrexed, continues through both chemotherapy visits and for 21 days after the last pemetrexed dose. B12 is given in the preceding week. The source repeats B12 every three cycles if pemetrexed continues; this planned course has only two pemetrexed administrations, so do not automatically continue B12 in immune-only cycles. Your team reconciles the steroid used to prevent pemetrexed rash with the anti-sickness steroid into one clear prescription. Do not add the two schedules together or use leftover steroids yourself. Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose.

2 Cycle 2 onwards Day 1 · nivolumab + ipilimumab every 42 days

Ipilimumab is low risk and nivolumab minimal; this combined visit is low risk. No further pemetrexed or platinum: stop/review induction antiemetics and rash steroid; complete the folate tail only. Continue immune symptom education, bloods each treatment and glucose/TFT each 42-day cycle.

3 Cycle 2 onwards Day 22 · nivolumab alone, between ipilimumab visits

No routine prophylactic antiemetic course for nivolumab-only minimal-risk treatment; prescribe symptom-directed rescue under local policy if needed. Ipilimumab is due at the next 42-day cycle Day 1, not every nivolumab visit. Immune toxicity can still occur between ipilimumab visits; the joint hold rule remains relevant.

Effects around the chemotherapy visits

These are selected possibilities, not a forecast or a complete medicine leaflet.

Sickness, appetite and constipation

Take preventive anti-sickness medicines as prescribed. Tell the team if they do not control symptoms. Persistent vomiting, difficulty keeping fluids down or much less urine need prompt advice. Some anti-sickness and pain medicines cause constipation; ask for a bowel plan.

Blood counts and infection

Fever, shivers or suddenly feeling unwell need immediate oncology advice, even with a normal temperature. Counts can fall between visits and cannot be judged from the calendar. Report unexpected bruising, bleeding or worsening breathlessness.

Mouth, skin and tiredness

Use gentle mouth care and report ulcers, painful swallowing, rash or reduced intake. Balance rest with manageable activity. Marked or sudden weakness needs assessment: fatigue can also be caused by infection, anaemia or immune hormone problems.

Kidneys, hearing and nerves

Cisplatin can injure kidneys and affect blood salts, hearing and nerves. Fluids before and after the infusion reduce particular risks, and your fluid advice is individual. Tell the team about reduced urine, new ringing in the ears, hearing loss, tingling or numbness before another chemotherapy dose.

Immune effects need early reporting



Breathing, bowel, skin and urine

New cough or breathlessness, diarrhoea, blood or mucus in stool, severe abdominal pain, yellow eyes, dark urine, reduced urine or swelling need prompt assessment. Do not assume these are ordinary chemotherapy effects. Blistering or peeling skin or sores in the eyes or mouth need urgent care.

Hormones, heart, muscles and nerves

Report unusual weakness, persistent headache, dizziness, marked thirst, frequent urination, palpitations, muscle weakness or vision changes. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care. Some hormone effects require long-term replacement.

Treatment may need to pause

The team may withhold both immune medicines, investigate and prescribe steroids or other treatment. This is different from the short preventive steroid course around chemotherapy. Do not start leftover steroids, change their dose or abruptly stop a prescribed course.

Living with treatment

The aim is to fit care around the things that matter to you.

Food, fluids, work and movement

Ask for help with weight loss, reduced appetite, work adjustments, transport or household tasks. A fixed fluid target is not suitable for everyone. Do not drive if dizzy, very tired or affected by sedating medicines; cisplatin anti-sickness treatment can include olanzapine.

Fertility, pregnancy and breastfeeding

Discuss fertility before treatment where relevant. Ask for a written contraception plan covering all four medicines and the period after treatment. Report possible pregnancy promptly and do not breastfeed unless your team confirms it is safe.

Support and choices

Ask for an interpreter, accessible information, psychological support or a trusted person to join appointments. Discuss alternatives, expected benefits and lasting harms. You can revisit your preferences as treatment continues.

Questions worth asking

Will chemotherapy continue for months?

The planned pemetrexed/platinum course is two administrations, on Days 1 and 22 of Cycle 1. Immune treatment can continue afterwards with a different support plan.

Why is ipilimumab absent from some visits?

Nivolumab is every 21 days; ipilimumab every 42 days. Day 22 has no ipilimumab. A withheld immune dose is a separate safety decision requiring team review.

How will we know whether it is helping?

Ask about the goals, scan plan and what would lead to continuing, changing or stopping. Side effects do not reliably show treatment response.

Can I decide from this guide whether treatment is safe today?

No. Your team must interpret symptoms, bloods, previous reactions and the current prescribing protocol. Omitted results are not presumed normal.

What if I miss folic acid or a steroid dose?

Contact your treatment team or pharmacy for an individual plan. Do not double doses or combine separate steroid schedules yourself.

Which risks continue after treatment?

Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose. Ask about follow-up for any lasting nerve, hearing, kidney, hormone or other problems.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Nivolumab

- Immune inflammation in multiple organs
- Infusion reactions
- Delayed effects after treatment stops

Ipilimumab

- Immune bowel, liver, skin and hormone inflammation
- Other serious organ inflammation

Pemetrexed

- Low blood counts and infection
- Mouth soreness, rash and fatigue
- Kidney toxicity

Cisplatin

- Nausea and vomiting
- Kidney, hearing and nerve effects
- Allergic reactions

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Active medicines: nivolumab (Days 1 and 22) + ipilimumab (Day 1 only) + pemetrexed (Days 1 and 22) + cisplatin (Days 1 and 22)

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on treatment day; 80 mg orally on each of the following two days	Repeat the three-day course separately around Cycle 1 Day 1 and Day 22
Ondansetron	16 mg orally	Each chemotherapy treatment day Review QT risk, interactions and individual dose choice.
Antiemetic dexamethasone reference	12 mg orally on treatment day; 8 mg orally daily for the next three days	Repeat around both chemotherapy visits Reconcile with pemetrexed rash steroids and aprepitant interaction into ONE prescription; do not add schedules.
Olanzapine	5 mg orally daily	Treatment day and the next three days, around each chemotherapy visit Generic high-risk NCIS row; consider sedation and driving, and extrapyramidal effects with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row does not define an end day or PRN qualifier; prescriber must specify duration, suitability and interaction risks.

Medicine	Dose / route	When
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout pemetrexed and for 21 days after its last dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed; source repeat every three cycles only while pemetrexed continues This induction contains two pemetrexed administrations only. Do not automatically continue B12 during immune-only treatment.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, treatment day and day after each pemetrexed administration Reference requirement: reconcile with antiemetic steroid into one clear prescription, not two added courses.

Additional phase-specific support

- Cycle 1: nivolumab, pemetrexed and cisplatin on Days 1 and 22; ipilimumab on Day 1 ONLY. Day 22 has no ipilimumab.
- Use generic high-risk NCIS row including olanzapine. Cisplatin requires pre/post hydration and electrolyte review; follow individual/local fluid policy.
- Folic acid starts before pemetrexed, continues through both chemotherapy visits and for 21 days after the last pemetrexed dose. B12 is given in the preceding week. The source repeats B12 every three cycles if pemetrexed continues; this planned course has only two pemetrexed administrations, so do not automatically continue B12 in immune-only cycles.
- Your team reconciles the steroid used to prevent pemetrexed rash with the anti-sickness steroid into one clear prescription. Do not add the two schedules together or use leftover steroids yourself.
- Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Cycle 2 onwards Day 1 · nivolumab + ipilimumab every 42 days

Active medicines: nivolumab + ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	Day 1 before treatment, if following the NCIS low-risk default Individualise/local policy. No automatic antiemetic corticosteroid course.

Medicine	Dose / route	When
Metoclopramide	10 mg orally three times daily as needed	Subsequent days as prescribed Specify duration and check contraindications/interactions.

Additional phase-specific support

- Ipilimumab is low risk and nivolumab minimal; this combined visit is low risk.
- No further pemetrexed or platinum: stop/review induction antiemetics and rash steroid; complete the folate tail only.
- Continue immune symptom education, bloods each treatment and glucose/thyroid function tests (TFT) each 42-day cycle.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Cycle 2 onwards Day 22 · nivolumab alone, between ipilimumab visits

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine prophylactic antiemetic course for nivolumab-only minimal-risk treatment; prescribe symptom-directed rescue under local policy if needed.
- Ipilimumab is due at the next 42-day cycle Day 1, not every nivolumab visit.
- Immune toxicity can still occur between ipilimumab visits; the joint hold rule remains relevant.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Nivolumab, ipilimumab, pemetrexed and cisplatin

The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and cisplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle. Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hormones / brain

Nivolumab / ipilimumab

Unusual weakness, headache, dizziness, thirst or passing much more urine.

2 Lungs

Nivolumab / ipilimumab

New cough or breathlessness needs prompt assessment; infection or cancer can cause similar symptoms.

3 Heart / muscles

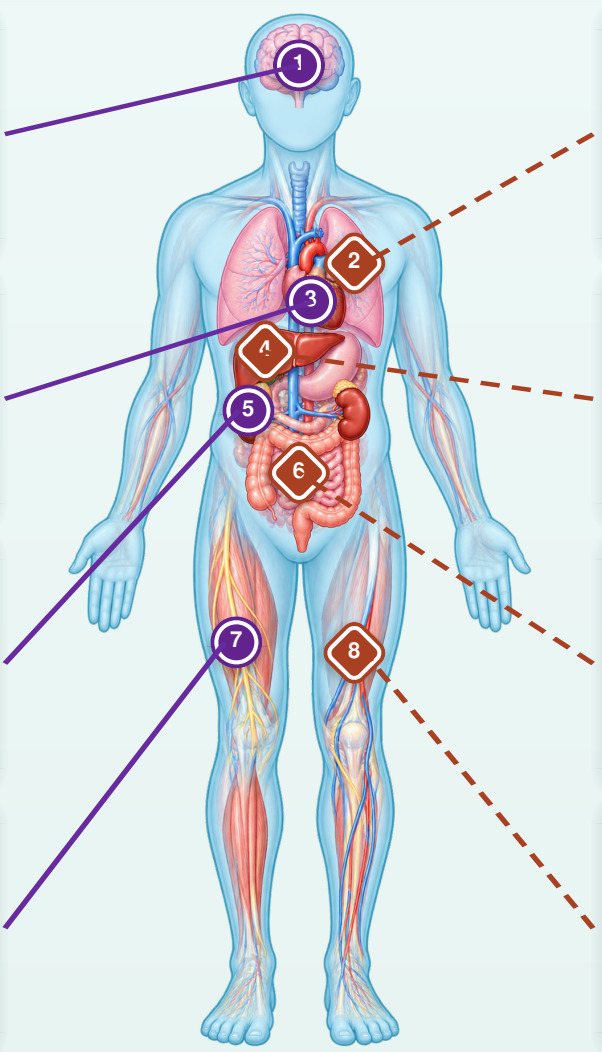
Nivolumab / ipilimumab

Chest pain, racing heartbeat, fainting or new muscle weakness needs urgent assessment.

4 Liver

Nivolumab / ipilimumab

Yellow eyes/skin, dark urine or abnormal tests can signal inflammation.



5 Kidneys

Pemetrexed / Cisplatin / immune medicines

Kidney tests matter; report less urine, swelling or difficulty drinking.

6 Bowel / mouth

Pemetrexed / nivolumab / ipilimumab

Report new diarrhoea, pain, blood/mucus or mouth ulcers; do not assume ordinary chemotherapy upset.

7 Skin / nerves / hearing

Immune medicines / Cisplatin

Report spreading rash, blistering, weakness, numbness or ringing/hearing changes.

8 Blood / infection

Pemetrexed / Cisplatin

Fever, shivers, sudden illness or unusual bleeding: contact oncology immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Follow your oncology team's emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00714 v4 · official regimen**
   https://healthservice.hse.ie/documents/6876/714_V4_Nivolumab_ipili_plus_PEMEtrexed-CISplatin.pdf
2. **CheckMate 9LA final six-year outcomes**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC12166824/
3. **EMA nivolumab**
   https://www.ema.europa.eu/en/medicines/human/EPAR/opdivo
4. **EMA ipilimumab**
   https://www.ema.europa.eu/en/medicines/human/EPAR/yervoy
5. **EMA pemetrexed**
   https://www.ema.europa.eu/en/medicines/human/EPAR/alimta
6. **NCCP NCIS Medical Oncology antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
7. **HSE · chemotherapy safety and when to contact your team**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **EMA Opdivo current product information (checked 29 October 2026)**
   https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf